FLUPENTIXOL + MELITRACEN

Not approved in many countries

Avoid long-term

CHEMICAL NAME Flupentixol: (Z)-2-[4-[3-[2-(trifluoromethyl)thioxanthen-9-ylidene]propyl]piperazin-1-yl]ethanol + melitracen: 3-(10,10-dimethylanthracen-9(10H)-ylidene)-N,N-dimethylpropan-1-amine

CLASS Combination: thioxanthene antipsychotic (low dose) + tricyclic antidepressant

TRADE NAMES Deanxit · Franxit · Danxi

THERAPEUTIC USES / INDICATIONS

- Mild–moderate mixed anxiety and depression (psychosomatic symptoms) — widely used in Middle East/Asia.
- Short-term use when other treatments unsuitable.

ADMINISTRATION

- **PO** in the morning (and midday) — not in the evening (activating).
- Each tablet: flupentixol 0.5 mg + melitracen 10 mg.

DOSAGES

- **Adult:** 2 tablets daily (morning and noon); in severe cases the morning dose may be 2 tablets; maintenance usually 1 tablet in the morning.
- **Elderly:** 1 tablet in the morning.
- Use for short periods; taper to stop.

PHARMACOKINETICS

- **Peak:** flupentixol 4 h; melitracen 4 h.
- Metabolised in liver.
- **t½:** flupentixol ~35 h; melitracen ~19 h.

NURSING MANAGEMENT

- Assess mood and anxiety; review need regularly (avoid long-term).
- Monitor for involuntary movements, restlessness.
- Pulse and BP; cardiac history.

PHARMACODYNAMICS

- Flupentixol (low dose): **dopamine D1/D2** antagonism with anxiolytic, activating effects.
- Melitracen: **NA and 5-HT reuptake** inhibition (TCA).

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Restlessness, insomnia (if taken late).
- Dry mouth, constipation, blurred vision.
- Dizziness, tremor.

ADVERSE EFFECTS (SERIOUS)

- Extrapyramidal symptoms, tardive dyskinesia (long-term).
- Cardiac arrhythmias (TCA component).
- Neuroleptic malignant syndrome (rare); suicidality.

CONTRAINDICATIONS & CAUTIONS

- Recent MI, heart block, arrhythmias.
- Circulatory collapse, CNS depression, coma.
- Untreated angle-closure glaucoma; MAOI use.
- Severely excited or agitated patients.

KEY INTERACTIONS

- **MAOIs:** hypertensive crisis.
- **Alcohol, sedatives:** ↑ CNS depression.
- **QT drugs, anticholinergics:** additive toxicity.

PATIENT EDUCATION

- Take in the morning (and early afternoon if prescribed), not at night.
- Report abnormal movements, restlessness or palpitations.
- Not for long-term use without review.

KEY POINTS

- Antipsychotic + TCA combination (Deanxit).
- EPS/tardive dyskinesia with long-term use.
- 1–2 tablets in the morning.
- Avoid in cardiac disease.

SOURCES CHECKED [795] Deanxit product information (HSA, Singapore)

Full references p. 432–457